

Sleep regimen for tonight

Action handout based on OE #26 (5/23) Tier 1 ranking · 2026-05-23

For Dad, Mom, Leah, David,
Debbi, family team

TWO REASONABLE OPTIONS FOR TONIGHT (PER OE #26 + OE #28 FOLLOW-UP)

The original OE #26 ranked lemborexant as Tier 1 and quetiapine as Tier 4. The OE #28 follow-up — surfacing Dad's documented 4-week tolerance of quetiapine 50 mg in 11/2025 plus the PCP's not-escalating posture on the cardiac findings (12+ months without refreshing the EKG) — shifted quetiapine to Tier 2–3. **Both options below are now reasonable; the choice for tonight is closer to a nocebo-vs-cardiac-caution tradeoff than a clear winner.**

Option A — Lemborexant 10 mg single dose + melatonin 5 mg HS + lorazepam 0.5 mg PRN rescue

Lemborexant 10 mg as a SINGLE dose at bedtime (not split like last night), on empty stomach, ≥ 7 hours of bedtime remaining. **Melatonin reduced from 10 mg to 5 mg. Lorazepam 0.5 mg at bedside as PRN rescue only** — not co-administered; take only if not asleep within ~ 90 min. Fall precautions.

Pros: avoids cardiac/Lewy concerns entirely; cleanest pre-ECT profile; no D2 receptor activity. *Cons:* not yet adequately trialed in Dad (last night's split-dose failure created a "Dayvigo didn't work" expectation); meaningful nocebo risk on retry.

Option B — Quetiapine 25 mg HS + melatonin 5 mg HS

Quetiapine 25 mg HS as a single dose at bedtime. Melatonin reduced from 10 mg to 5 mg.

Pros: known-tolerated (4 weeks at 50 mg in 11/2025 with same cardiac substrate, same concurrent duloxetine, no adverse events); sedation IS the desired effect tonight; sidesteps the Dayvigo nocebo loop; PCP has not escalated the cardiac findings in 12+ months. *Cons:* residual nordoxepin (still on board through $\sim 5/26$) adds a QT-active variable not present during the 11/2025 trial; lowest-effective-dose principle in possible prodromal Lewy-spectrum disease argues for 25 mg over 50 mg for tonight; a fresh ECG would be ideal if conveniently obtainable.

If Option B is chosen, 25 mg HS is preferred over 50 mg for tonight given (a) the nordoxepin variable not previously tested, (b) lowest-effective-dose principle, (c) the option to escalate to 50 mg on Night 2 if 25 mg is insufficient — given prior tolerance at that dose. **If Dr. L proposed 50 mg directly, asking for 25 mg as the entry dose is reasonable.**

Why this regimen, not the others

WHY LEMBorexant 10 MG SINGLE DOSE (NOT SPLIT)

Last night's lemborexant 10 mg total taken as half-tablet (5 mg) + later half-tablet (5 mg) was pharmacokinetically suboptimal. Lemborexant has a half-life of ~18 hours and is designed to be taken as a single dose immediately before bedtime; the FDA label specifies single-dose administration. The first 5 mg partially cleared before the second 5 mg achieved peak; neither dose alone reached the receptor occupancy of a single 10 mg bolus. **Lemborexant has not been adequately trialed yet** — last night's failure shouldn't be read as the drug failing for Dad. Lemborexant carries the lowest fall risk (no postural-stability impairment in elderly studies) and the cleanest pre-ECT profile of the options.

WHY MELATONIN 5 MG RATHER THAN 10 MG

Evidence for melatonin benefit above 5 mg in elderly is absent; doses above 5 mg are supraphysiologic with no demonstrated additional sleep benefit. Maintaining the 10 mg dose adds no advantage and may produce daytime grogginess.

WHY LORAZEPAM AT BEDSIDE AS RESCUE, NOT AT START OF NIGHT

Combining lemborexant + lorazepam at the start of the night risks (a) muddying the assessment of whether lemborexant works for Dad; (b) additive sedation/fall risk; (c) reinforcing a sleep-requires-Ativan pattern. Holding 0.5 mg in reserve as a rescue preserves the option without committing to the combination. One dose if truly needed at 90 minutes of non-sleep is appropriate; a second dose mid-night is not.

What to avoid tonight (and why)

DO NOT — OPTIONS THAT REMAIN CLEARLY WORSE FOR TONIGHT

- **DO NOT use quetiapine 50 mg as tonight's entry dose** — even though Dad tolerated 50 mg in 11/2025 and 50 mg would be defensible based on prior tolerance, the nordoxepin residual (on board through ~5/26) wasn't present during the 11/2025 trial, and the lowest-effective-dose principle applies in possible prodromal Lewy-spectrum disease. Start at 25 mg HS; escalate to 50 mg on Night 2 if 25 mg is insufficient. (Important nuance: this is not "Seroquel is dangerous" — it's "25 mg is the right entry dose tonight; 50 mg is fine on Night 2 if needed.")
- **DO NOT re-introduce doxepin tonight.** Nordoxepin (active metabolite, 31-hour half-life) is still meaningfully on board through ~5/26 from the recent ~17-night course. Re-introducing 6 mg would add to the residual, potentially producing supra-therapeutic exposure. Also: FDA geriatric max is 3 mg, not 6 mg. If doxepin is desired later, wait until ~5/28 and start at 3 mg.
- **DO NOT escalate to olanzapine if the chosen option fails** — given the 4/2026 near-panic-attack rebound experience and pending Lewy-spectrum workup. Olanzapine has higher D2 affinity than quetiapine (~70x — Ki ~11 nM vs ~770 nM) and substantially higher parkinsonism risk, and is categorically avoided in confirmed DLB per Lancet Neurology 2025.
- **DO NOT combine lemborexant + lorazepam at the start of the night** if Option A is chosen — lorazepam is rescue-only, not co-administered.
- **DO NOT combine lemborexant and quetiapine the same night** — these are alternatives, not stack-mates. Pick one option; commit to it for tonight; reassess in the morning.

WHAT CHANGED FROM THE INITIAL OE #26 RANKING

The initial OE #26 (5/23 morning) ranked lemborexant Tier 1 and quetiapine Tier 4, driven by population-level chronic-use safety signals (Hui 2025: HR 3.1 mortality / HR 8.1 dementia / HR 2.8 falls vs trazodone; Højlund 2022: aHR 1.52 MACE). The OE #28 follow-up shifted this: **(a)** Dad tolerated quetiapine 50 mg × 4 weeks in 11/2025 with the same cardiac substrate and same concurrent duloxetine; **(b)** the PCP has not refreshed the EKG or escalated the cardiac findings in 12+ months, suggesting the findings are stable and not blocking standard prescribing; **(c)** the chronic-use HRs from Hui 2025 and Højlund 2022 are months-of-exposure endpoints and should not be the primary driver for a 2–3 night bridge; **(d)** the HR 8.1 for dementia almost certainly overestimates the causal effect due to residual confounding-by-indication (patients prescribed quetiapine for elderly insomnia are enriched for pre-existing cognitive complaints that themselves are early prodromal-dementia markers); **(e)** fresh ECG is ideal but not an absolute barrier given prior tolerance. The "Option B is contraindicated" framing in the initial OE was over-calibrated for Dad's specific case.

OE #28 second follow-up (5/23 PM) added three further specifics: **(f) Shu 2023 FAERS analysis** — cardiac AEs from quetiapine are "early failure type" / front-loaded (median time-to-onset for moderate cardiac AEs = 0 days; IQR 0–8). Practical implication: having cleared the highest-risk first-exposure window at 50 mg in 11/2025, re-exposure at 25 mg carries substantially lower acute QT risk than initial exposure did. **(g) Sleep-deprivation QT impact quantified** — Aldana 2025 + Browne 1983 + Joukar 2013 → modest shift in QTcF from 401 to estimated 406–421 ms with current sleep crisis; still well below 500 ms danger threshold; ~80–100 ms headroom remains. **(h) Saturday ECG pathway practical** — Michigan Medicine urgent care (12-lead in 1–2 hours) is the most efficient route; KardiaMobile 6L as interim if family has one. Worth doing if convenient; not an absolute gate.

Decision tree: do they need an ECG before tonight?

IF CHOOSING OPTION A (LEMBOREXANT) — ECG NOT RELEVANT

Lemborexant has no cardiac considerations. Proceed directly with Option A tonight; no ECG needed.

IF CHOOSING OPTION B (QUETIAPINE 25 MG) — THE URGENT CARE ECG IS THE RECOMMENDED NEXT STEP IF IT'S STILL DAYTIME

If it's still daytime AND urgent care can be reached in the next 1–2 hours: go get the ECG before tonight's dose. This is the recommended path. The clinical indication to give the urgent care provider is: *"76-year-old male with LAFB, sinus bradycardia, borderline 1° AV block, on duloxetine, considering quetiapine initiation, last ECG over 1 year ago — needs 12-lead ECG for QTc assessment before starting."* Michigan Medicine urgent care is the most efficient route; typically 1–2 hours; results immediate.

If it's already late and urgent care would push tonight's dose past a reasonable bedtime: proceed with 25 mg quetiapine without the fresh ECG. The prior tolerance evidence (50 mg × 4 weeks in 11/2025 with same cardiac substrate + same concurrent duloxetine + no adverse event) + the ~80–100 ms QTcF headroom (current estimated QTcF 406–421 with sleep-deprivation effects, well below 500 ms danger threshold) + the Shu 2023 finding that quetiapine cardiac AEs are front-loaded (and Dad already cleared the first-exposure window once) together provide sufficient clinical confidence at 25 mg. The ECG would have been reassurance, not a prerequisite — getting an ECG next week is still worthwhile but doesn't need to gate tonight's regimen.

Interim option if family has a KardiaMobile 6L device: provides automated QTc with limitations; useful as interim reassurance but not a full substitute for 12-lead.

Not the right path: ED visit solely for the ECG — slower and lower-value than urgent care.

Fallback for tomorrow night if tonight fails

IF OPTION A (LEMBOREXANT) FAILS TONIGHT

Option 1: Switch to quetiapine 25 mg HS for Night 2 (per OE #28 follow-up — known-tolerated, sleep-indication match). **Option 2:** Trazodone 50 mg + lorazepam 0.5 mg + melatonin 5 mg HS (the OE #26 Tier-1-alt — GABAergic + histaminergic). Both are reasonable.

IF OPTION B (QUETIAPINE 25 MG) FAILS TONIGHT

Escalate to quetiapine 50 mg HS for Night 2 — defensible based on the documented 4-week tolerance of 50 mg in 11/2025. Or switch to single-dose lemborexant 10 mg with the explicit framing for Dad that last night's failure was the split-dose, not the drug.

Monitoring tonight

- Note time lemborexant taken; time of sleep onset; any awakenings; total sleep time.
- If lorazepam rescue used: note time, dose, and effect.

- Watch for excessive sedation, confusion, or unsteadiness — especially if lorazepam is used.
- Watch for the shaking pattern overnight or AM — does it change with the regimen?
- Note any new dream content or RBD-like episodes (Dayvigo increases REM sleep; theoretical concern for RBD-spectrum patients).

Sequencing and failure criteria

IF OPTION A (LEMBOREXANT) IS CHOSEN — COMMIT TO 2–3 NIGHTS AS A FAIR TRIAL

Pharmacological rationale for sticking with lemborexant for 2–3 nights: lemborexant has a 17–19 hour half-life and accumulates with daily dosing; plasma levels on Night 2 are ~50% higher than Night 1, and by Night 3–4 you're approaching steady state. Combined with: partial sleep gain on Night 1 (even if modest) reducing homeostatic pressure for Night 2; reduced "is this going to work" anxiety after some sleep is achieved; and the OE #23 medium-term-sleep-regimen finding that lemborexant requires a 1–2 week trial to assess. **Switching after one suboptimal night doesn't give the agent a fair pharmacokinetic chance.**

Concrete criteria for Night 2 decision if Option A chosen:

- **If Night 1 sleep ≥ 4 hours:** retry same regimen Night 2 — accumulation effect expected to help.
- **If Night 1 sleep 2–4 hours:** retry same regimen Night 2 (fair-trial principle) but consider adding lorazepam 0.5 mg HS as a scheduled adjunct (not just PRN) to provide GABAergic floor while the orexin blockade builds up.
- **If Night 1 sleep < 2 hours (essentially zero):** switch to Option B (quetiapine 25 mg HS) for Night 2 — at that point the cumulative sleep crisis is dangerous enough that the known-tolerated agent's floor wins over the fair-trial principle.
- **If Night 1 sleep ≥ 6 hours:** working regimen, hold steady; reassess after 3–4 nights.

IF OPTION B (QUETIAPINE 25 MG) IS CHOSEN — NIGHT 2 DECISION IS MORE DOSE-TITRATION THAN AGENT-SWITCH

Concrete criteria for Night 2 decision if Option B chosen:

- **If Night 1 sleep ≥ 6 hours:** hold at 25 mg HS.
- **If Night 1 sleep 3–5 hours:** escalate to 50 mg HS for Night 2 (defensible based on the 11/2025 prior tolerance at 50 mg $\times$ 4 weeks).
- **If Night 1 sleep < 3 hours:** something is meaningfully different from the 11/2025 trial. Consider switching to Option A (lemborexant 10 mg single dose) for Night 2, with explicit framing that the prior split-dose was the failure mode.

If Mom needs to talk to Dad about tonight's plan

FOR MOM TO DAD (OPTION A — LEMBOREXANT)

"Tonight we're going to take the Dayvigo as a single tablet at bedtime — not split like last night. Last night's split-dose didn't give the medicine a fair chance to work. We have Ativan as a backup if you're not asleep by 90 minutes; one dose tonight is fine and isn't a habit problem. One night doesn't tell us much — if tonight isn't great, we have a clear plan for tomorrow."

FOR MOM TO DAD (OPTION B — QUETIAPINE 25 MG)

"Tonight we're going to use Seroquel 25 mg — half the dose you were on last fall, which you tolerated fine for about a month. That earlier trial just didn't help with the anxiety, which isn't what we need it for now; tonight we need help sleeping, and the sedation part is exactly what helps with that. We can go up to 50 mg tomorrow night if 25 mg isn't enough. Mom can come hold your legs if the shaking gets bad."

This handout reflects the OE #26 (5/23) Tier 1 ranking based on FDA labels, the SUNRISE-1/SUNRISE-2 trials (lemborexant in elderly), the Pariwatcharakul 2025 dose-dependent benzodiazepine-ECT analysis, the Hui 2025 quetiapine-in-elderly safety analysis, the Wang 2024 QT-prolongation data, the Højlund 2022 MACE analysis, Lancet Neurology 2020/2025 LBD landscape reviews, the doxepin / nordoxepin FDA label, and the De Crescenzo 2022 Lancet network meta-analysis. The recommendations are interpreted for Dad's specific profile (age 76, severe TRD, ≥ 4 cardiac QT risk factors, findings under evaluation for possible prodromal Lewy-spectrum disease, residual nordoxepin from recent doxepin course, activation-intolerance pattern, planned U-M ECT 1–3 weeks out).